

Synthesis

Your sources. A wiki that lasts.

Selected evidence → durable knowledge

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Sources persist. Does the understanding?

CONNECTIONS

Across sources

CONTEXT

Across time

EVIDENCE

Back to origin

| Knowledge should compound.

The LLM Wiki idea

Inspired by [Andrej Karpathy's LLM Wiki pattern](#)

PAGES

Give topics a home

LINKS

Keep context
connected

UPDATES

Build on earlier
sources

| A synthesised wiki. Durable knowledge.

HACA 2025, in one wiki

66

recordings

344

draft wiki pages

29

later-source updates

Automatic compilation - publication consistency pass

Three talks. One wiki page.

PRODUCT

Users and ownership

MODEL

Probabilistic demand

DELIVERY

One trust → seven

| Demand and capacity modelling for acute care

Synthesis

Local AI - ready

Search your knowledge base...

Add source

Vault tools

WORKSPACE

Wiki

Review

Synthesis review99+

Ask wiki

MANAGE

Sources

Wiki health

KNOWLEDGE

Compiled wiki

WIKI PAGES344

A&E waiting time inequality and multivariate analysis

Accessibility testing for Shiny applications

ADASS Strategic Framework and Data Hub

Adult social care data challenges

Agile adaptation in data science

AI as a systems-level technology in healthcare

AI capabilities, misconceptions, and future potential

AI efficiency versus equity and nuance

AI integration for PDF data extraction

AI-assisted evidence screening methodology

AI-Assisted Transition from Excel to Python

Alastair McAlpine and Scottish Government Statistics

Analyst professional development strategies

Analyst Role in Digital Transformation

Analyst X Tool and Training Ecosystem

Annual Analytical Plans

Arthritis UK lived experience

PROVENANCE

Evidence & context

3sources

4cited claims

9reviewed links

CLAIM EVIDENCE

1.

A simulation model was developed under the New Hospital Programme, managed by the Strategy Unit, to predict demand for acute care in the UK health and care system, with a focus on the period around 2041. The initiative aims to inform the construction of billion-pound hospitals by providing robust, evidence-based projections. A core ethos of the programme is that the modelling work is owned by the NHS for the NHS, rather than relying on external companies, to ensure public benefit and transparency. The project has received recognition for its approach, though specific award details are not elaborated.

[HACA2025 - Day 2 - Targeting Care/Advancing Analytics](#)

[HACA2025 - Day 2 - Targeting Care/Advancing Analytics](#)

[HACA 2025: Shift to Community: A model is for life, not just for Christmas...](#)

► Read full claim

2.

The programme employs a probabilistic modelling framework rather than relying on point estimates, arguing that point estimates are inherently flawed in the face of future uncertainty. This approach ensures that the analysis remains in the public record and can be scrutinised. The modelling process is collaborative, emphasising 'modelling with' stakeholders rather than 'modelling to' them, which is considered essential for high-stakes decisions such as hospital construction.

[HACA2025 - Day 2 - Targeting Care/Advancing Analytics](#)

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► Read full claim

3.

The project involved a phased rollout, expanding from initial testing with a single individual and one trust to deployment across seven trusts. It operated under a strict deadline, with a target date of 7 December for the model to go live. The rollout encountered significant operational difficulties. Internal staff were stressed, and external stakeholders, specifically the hospital trusts, were often unwelcoming of the standardized model. These challenges necessitated a shift in management strategy to ensure delivery despite resistance and chaotic conditions.

[HACA2025 - Day 2 - Targeting Care/Advancing Analytics](#)

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COMPILED WIKI PAGE

Demand and capacity modeling for acute care

3 supporting sources · 4 cited claims

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The project involves extensive analytical work, including the creation of reports and dashboards to help users understand the data and interpret the results. The complexity of the data and results necessitated significant effort to assist users in setting parameters and understanding the inputs and outputs of the model. However, initial user feedback was ad hoc, and the prioritization of developments was not formally structured. This lack of structure led to issues with rollout and missed opportunities, illustrating the consequences of not fully integrating product management principles and user-centric design in complex analytical projects. The case underscores the importance of engaging

— Reviewed wiki link --- Mutual semantic proximity ● Direct search result

"demand capacity" · 14 matching pages · 81 directly connected pages

Clear search

Titles appear as space allows. Select a page to explore its neighbours; open it to inspect evidence. Proximity suggests similarity, not proof. Axes, rotation, and distances between disconnected groups have no meaning.



Connected pages

Focused on "Demand and capacity modeling for acute care"
· 24 visible connections

Open page

Clear focus

Filter page titles

24 of 24 pages

Capacity challenges in renal services
Reviewed wiki link

CKD progression modeling methodology
Reviewed wiki link

Data Infrastructure and Versioning Challenges
Reviewed wiki link

Framework for quantifying potentially mitigable hospital capacity
Search match · Reviewed wiki link

Cross-source synthesis

Similarity finds possible neighbours → **LLM comparison** proposes how they may relate → **Human review** decides what becomes a wiki link

Model confidence: 82% · llm_cross_source_review · glm-4.7-flash:latest. This is not evidential certainty.

Supporting wiki pages

[Chief Data and Analytics Officer Network](#)

[The value of data analytics in healthcare](#)

Supporting sources

[HACA2025 - DAY 2 - Main Stage - Opening Session](#)

[HACA 2025 - Day 1 - Main Stage](#)

Investigate

Reject

Confirm link

Confirmation records this typed reviewed relationship and an ordinary explicit wiki link.

Lower blood pressure. Better outcomes?

Type 2 diabetes · systolic target <120 vs <140 mm Hg

EXISTING WIKI

ACCORD BP:
No significant
reduction in major
cardiovascular events.



NEW SOURCE

BPROAD:
Fewer major
cardiovascular events.



PROPOSED WIKI UPDATE

Keep both findings.
Preserve their trial
context.

| New evidence can qualify—not overwrite—the wiki.

Supported inputs: pasted text · Markdown or text files · text PDFs · YouTube videos or playlists

WORKSPACE

Wiki

Review1

Synthesis review

Ask wiki

MANAGE

Sources

Wiki health

HUMAN REVIEW

Review proposed changes

AI proposals remain separate from the compiled wiki until every change has an explicit decision.

Review every change before deciding.

PENDING SOURCES1

Curated extract:
BPROAD (2024)

1 proposed change

SOURCE PROPOSAL

Curated extract: BPROAD (2024)

text · 1 proposed change

In Chinese adults with type 2 diabetes and increased cardiovascular risk, BPROAD found fewer major cardiovascular events with intensive control, with more symptomatic hypotension and hyperkalaemia.

Inspect source

Review the proposal before applying it.
Choose Include or Exclude for each change. Included text can be edited before the final Apply step.

Proposed changes

Include all changes

CONTRADICT

Blood-pressure targets across trials

synthesis

Decision

Decision required

Include in wiki

Exclude from approval

SOURCE CONTEXT & PROVENANCE

Curated extract: BPROAD (2024)

Source summary: fewer major cardiovascular events; more symptomatic hypotension and hyperkalaemia.

CURRENT DURABLE PAGE

ACCORD BP: no significant reduction in its primary cardiovascular composite in adults with type 2 diabetes; stroke was less frequent.

PROPOSED EVOLVED PAGE

Keep ACCORD BP and add BPROAD’s later finding. Preserve population, outcome definitions and harms rather than choosing a universal answer.

1 decision remaining · 0 include · 0 exclude

Reject proposal

Review 1 remaining

The wiki is yours to keep

READ

Ordinary Markdown

TRACE

Sources and history

RECOVER

Export · rebuild · undo

Linked, revisable pages have endured at scale: Wikipedia, 2001 → 65 million articles. [Wikimedia Foundation](#)

Current scope

SINGLE-USER

Local-first MVP

HUMAN JUDGEMENT

Check claims and
links

PROVIDER CHOICE

Remote AI receives
content

Research and knowledge work. No clinical decision support.

Questions

Where would a maintained wiki improve real work?

<https://github.com/The-Strategy-Unit/synthesis>

Q&A DETAIL

Appendix topics

GRAPH

How to read the graph

REVIEW

Full wiki link review

PORTABILITY

Vault and recovery

Q&A DETAIL

How to read the graph

What you see	What it means
Focused page	A named neighbourhood to explore
Solid blue line	Durable wiki link
Dashed grey line	Semantic suggestion; not yet a wiki link
Position or distance	Navigation layout; not importance, proof or certainty

Connections suggest routes to inspect. Evidence remains the test.

Synthesis

Local AI - ready

Demand and capacity

WORKSPACE

Wiki

Review

Synthesis review99+

Ask wiki

MANAGE

Sources

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KNOWLEDGE

Compiled wiki

SEARCH RESULTS

Keyword search · highest rank

Demand and capacity

Keyword rank 1

Cross-source synthesis

×

Synthesis systematically evaluates plausible page pairs from different sources and remembers its progress. Proposed relationships and possible consolidations are hypotheses for review. Similarity and model confidence are not evidence, and no link is added automatically.

Filter proposals

Chief Data and Analytics Officer Network

Relationship

All relationships

Compare all sources

Select current filter

Clear selection

0 selected

Reject selected

Confirm selected

Review the cited pages and sources before acting.

Supports

Chief Data and Analytics Officer Network ↔ The value of data analytics in healthcare

Supports

Chief Data and Analytics Officer Network ↔ Health and Care Analytics Conference (HACA)

Supports

Chief Data and Analytics Officer Network ↔ Structuring the NHS Analytic Community

Supports

Chief Data and Analytics Officer Network ↔ NHS Analytical Strategy and Workforce Challenges

Pending

Supports

Potential relationship

The 'Chief Data and Analytics Officer Network' page states that the network aims to ensure data expertise informs strategic decisions rather than being sidelined by clinicians or CIOs. This directly supports the 'The value of data analytics in healthcare' page, which argues that robust analytics are essential for making evidence-based decisions and countering reliance on 'policy based evidence' or beliefs. The network serves as a mechanism to operationalise the value of analytics by providing a platform for senior analytical professionals to advocate for data-driven decision-making.

Why it may matter

This relationship demonstrates how professional networks can operationalise the high-level advocacy for data analytics, providing a concrete mechanism for analysts to influence strategic decisions and ensure their expertise is not sidelined.

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Supporting wiki pages

[Chief Data and Analytics Officer Network](#)
[The value of data analytics in healthcare](#)

Supporting sources

[HACA2025 - DAY 2 - Main Stage - Opening Session](#)
[HACA 2025 - Day 1 - Main Stage](#)

Investigate

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Confirm link

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Evidence

Page

Connections

PROVENANCE

Evidence & context

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← Cross-source synthesis

Appendix topics Questions

Q&A DETAIL

Portable wiki, rebuildable catalogue

```
vault.json - schema.md
notes/      wiki pages
sources/    immutable evidence
history/     accepted updates and undo receipts

synthesis.db rebuildable catalogue
```

Export excludes the database and keys. Undo is newest-ingest-only.